9.1 Fractures

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective • "My leg hurts badly when anyone touches it." • Gives the pain 8/10 **Objective** • Limb swollen and out of shape X-ray shows a closed fracture of the tibia • Muscles in spasm	**ACTUAL** **Acute pain** related to bone fracture, soft-tissue injury and muscle spasm as evidenced by pain score 8/10 and guarding.	Pain 3/10 or less within 1 hour of the intervention.	• Before any movement, splint the limb as ordered. • Unless there is a reason not to, raise the limb above heart level. • Cold packs for the first 24–48 hours, as ordered. • Give analgesia as prescribed and check again. • When you move the limb, support it above and below the fracture.	Pain controlled: **goal met**.
Subjective • "I cannot walk or go to the toilet alone." **Objective** • Ordered non-weight-bearing • Affected limb moves very little	**ACTUAL** **Impaired physical mobility** related to fracture and prescribed immobilization as evidenced by inability to bear weight.	Within 3 days, gets about safely on crutches or a walker.	• Teach crutch walking together with the physiotherapist. • Encourage active exercise of the limbs that are not injured. • As allowed, teach isometric exercises for the injured limb. • Help with ADL as needed.	Moving about safely: **goal met**.
Subjective • "My toes feel tight and tingling." **Objective** • Swelling under the cast or splint • Closed fracture of the lower leg	**RISK** **Risk for peripheral neurovascular dysfunction** as evidenced by risk factors: fracture, swelling and immobilization device.	Limb stays warm and pink, with normal sensation, movement and capillary refill.	• Check the 5 Ps (pain, pallor, pulselessness, paresthesia, paralysis) every hour for the first 24 hours, then as ordered. • Pain out of proportion to the injury, or pain on passive stretch, must be reported at once. • Keep the limb raised and loosen the bandages if ordered.	Neurovascular status normal: **goal met**.